



Diagnostic Audit — Interview Guides

For Dr Debo Odulana & Tito Ipinmoye · hypothesis-driven, mapped to the MECE domains

Every interview is trying to **CONFIRM** or **KILL** a hypothesis, not just gather colour. Open collaboratively ("this is about how the place works, not about individuals; nothing is attributed to you"), listen more than you talk, and never make anyone feel blamed. Sharp questions are in red — use judgement on tone and timing.

Coverage is MECE by design: across the nine interviews, all ten domains are covered with no gaps — D1 Demand & Market, D2 Access & Patient Flow, D3 Clinical Care & Governance, D4 Pharmacy & Supply Chain, D5 Revenue Cycle, D6 Finance & Capital, D7 People & Organisation, D8 Infrastructure & Utilities, D9 Digital & Technology, D10 Governance & Compliance — each examined through the cross-cutting lenses (culture, cash, data, risk, stakeholder voice, scalability).

1. Owners & board (Mr Kabir Aregbesola, Mrs Abisodun Alli, Mr Ogochukwu Odum)

Lead: Debo

Domains: Demand & Market · Finance & Capital · People/Leadership · Governance & Compliance

Core

- What does success for Haven in 12 months look like, and what worries you most?
- Walk me through the money — what does a good month vs a bad month look like, and why?
- How are decisions made day to day, and who actually runs the facility operationally?
- How is the board constituted and how are owner decisions taken today?
- Appetite for a senior operations hire, and for the commission / incentive redesign?

Sharp / hypothesis-testing

- Is Haven a business that happens to do medicine, or a clinical practice that hasn't yet become a business? What in the numbers tells you which?
- If you had to dominate ONE segment in 18 months — premium paediatrics, HMO volume, or referral NICU — which, and does today's model serve it?
- Could the owners step away for a month and Haven still run? If not, what breaks?

2. Medical Director (Dr Odedina)

Lead: Debo + Tito

Domains: Clinical Care & Governance · Infrastructure/Equipment · People (clinical) · Risk & Safety

Core

- Talk me through clinical governance — protocols, incident review, M&M; how standards are set and checked.
- How ready is NICU to safely take more admissions? People, kit, protocols, cover — what's needed first?
- How is medical staffing structured — employed vs visiting consultants — and who is accountable to whom?
- Biomedical equipment and life support (incubators, ventilators, oxygen) — reliability, maintenance, backup?
- Power, oxygen and water resilience for NICU — what happens on an outage?

Sharp / hypothesis-testing

- Is safety currently a system, or a set of good individuals? What would fail if the best nurse were off?
- Would Haven survive a second serious adverse event during a growth push — clinically, reputationally, financially?
- Can you standardise safety and scale NICU on sessional consultants who run their own protocols?



3. Founder & clinician (Dr Shakirah Saliu)

Lead: Debo

Domains: Governance · Strategy & Vision · Clinical perspective · Culture (lens)

Core

- What was the original vision for Haven, and how do you feel it is tracking against it?
- As both an owner and a clinician, where do you see the biggest risks — clinical and operational?
- How are decisions taken among the owners, and where do you want Haven to be in two to three years?

Sharp / hypothesis-testing

- Are the owners genuinely aligned on what Haven is trying to win at, or pulling in different directions?
- Does how people are paid and recognised reward quality and ownership — or only activity and volume?

4. Nursing lead / Matron

Lead: Tito

Domains: Clinical Care & Governance · People & Organisation · Access & Patient Flow · Culture/Safety (lens)

Core

- Walk me through a shift start to finish — where does it break down?
- Which routines are meant to happen every shift, and which actually do? (handover, drug checks, emergency readiness)
- Staffing vs workload — where is it thin, and when?
- How is good or poor performance recognised or addressed on your team?

Sharp / hypothesis-testing

- If catching a safety problem slowed throughput, would a nurse be rewarded or penalised for stopping the line?
- When the person who 'knows how it works' is on leave, does the process survive — or does it depend on them?

5. Pharmacy lead

Lead: Tito

Domains: Pharmacy & Supply Chain · Finance (margin) · Working capital (lens)

Core

- How is stock ordered, received, stored, dispensed and counted? Walk me through it.
- How often do you stock out, of what, and what drives it?
- How is pharmacy margin worked out, and where is it lost between purchase and sale?
- What would a vendor-managed inventory model change for you?

Sharp / hypothesis-testing

- How much cash is sitting on the shelf right now that the business can't use — and who owns that number?
- Is the reported pharmacy 'profit' a real margin or just a markup? Show me a line.

6. Front desk / Customer service

Lead: Tito

Domains: Access & Patient Flow · Revenue Cycle (front-end) · Demand (experience, lens)

Core

- Walk me through a patient from arrival to billing — where do they wait, get frustrated, or fall through?
- Booking, deposits and HMO verification — how does it actually work?
- What do patients complain about most, and what happens to that feedback?

Sharp / hypothesis-testing

- Where in the front-end journey do we lose patients — or lose the claim before it's even submitted?



7. Accounts / Finance

Lead: Tito + Debo

Domains: Finance & Capital · Revenue Cycle · Digital & Data (reporting, lens)

Core

- How are the numbers produced — what system, what's automated, what's manual, what reconciliations run?
- Receivables: how are HMO claims tracked, chased and reconciled? Which HMOs, how aged?
- What in the current reporting do you NOT trust, and why?
- What is the cash-conversion cycle — how long from service delivered to cash in the bank?

Sharp / hypothesis-testing

- Can you produce a P&L; by service line, today, that foots? If NICU doubled tomorrow, would you know within a week if it was profitable?
- Which HMOs are actually profitable after denials, delay and cost-to-collect — and which should Haven fire?

8. Facilities / biomedical / IT

Lead: Tito

Domains: Infrastructure, Equipment & Utilities · Digital & Technology

Core

- Power, water and medical-gas (oxygen) supply — sources, backup, and what happens on failure?
- Biomedical equipment inventory — what's critical, is it maintained, calibrated, and is there redundancy?
- What clinical and billing systems (EMR/HMIS) are in use, who supports them, and what data can they export?

Sharp / hypothesis-testing

- If the grid dropped during a NICU resuscitation tonight, what exactly stands between that and a catastrophe?

9. Visiting consultants

Lead: Tito

Domains: People (consultants) · Clinical Care · Demand (they drive admissions)

Core

- What brings you to Haven, and what would make you bring more of your patients here?
- Are you bound by Haven's clinical protocols and safety routines, or do you run your own?
- How do the economics work for you — sessional fees, payment timeliness?

Sharp / hypothesis-testing

- What share of your admissions could move elsewhere tomorrow — and what would trigger that?
- If Haven asked you to follow its shift-level safety routines to the letter, would you — or would you push back?

A note on who's missing

Clinical leadership sits with the Medical Director (Dr Odedina, guide 2) and the nursing lead / matron (guide 4); Dr Shakirah is a founder-clinician, not the clinical lead. If Haven has a distinct Head of Operations, Head of Nursing, or HR/People lead, add a short guide — but the nine above already cover all ten domains, so any addition is depth, not a gap.